QUETIAPINE

Sedating

Minimal EPS — Parkinson's psychosis

CHEMICAL NAME 2-[2-(4-dibenzo[b,f][1,4]thiazepin-11-yl-piperazin-1-yl)ethoxy]ethanol (fumarate)

CLASS Second-generation (atypical) antipsychotic (dibenzothiazepine)

TRADE NAMES Seroquel · Seroquel XR · Ketipinor · Quetiapine (generic)

THERAPEUTIC USES / INDICATIONS

- Schizophrenia.
- Bipolar disorder: mania, bipolar depression, maintenance.
- Adjunct in major depression (XR).
- Psychosis in Parkinson's disease (off-label, low dose).

ADMINISTRATION

- **PO** IR twice daily or XR once daily in the evening (swallow whole, without food or light meal).
- Titrate over several days.

DOSAGES

- **Schizophrenia:** day 1 50 mg, day 2 100 mg, day 3 200 mg, day 4 300 mg; usual 300–450 mg/day (max 750 mg).
- **Mania:** up to 800 mg/day.
- **Bipolar depression:** 300 mg XR at bedtime.
- **Elderly:** start 25 mg, increase slowly.

PHARMACOKINETICS

- **Peak:** 1.5 h (XR 6 h).
- Metabolised by CYP3A4.
- **t½:** 7 h (active norquetiapine ~12 h).

PHARMACODYNAMICS

- **5-HT2A, D2** (low affinity, fast dissociation → minimal EPS), **H1** (sedation), **α1** (hypotension) antagonism.
- Norquetiapine inhibits noradrenaline reuptake (antidepressant).

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Sedation, dizziness, dry mouth.
- Postural hypotension.
- Weight gain, constipation.

ADVERSE EFFECTS (SERIOUS)

- Metabolic syndrome; ↑ cholesterol, triglycerides.
- QT prolongation; neutropenia.
- Suicidality (young people); misuse.
- ↑ stroke/mortality in elderly with dementia.

CONTRAINDICATIONS & CAUTIONS

- Use with strong CYP3A4 inhibitors (ketoconazole, ritonavir, clarithromycin).
- **Caution:** cardiovascular disease, elderly, diabetes.

KEY INTERACTIONS

- **CYP3A4 inhibitors:** ↑↑ levels.
- **Carbamazepine, phenytoin:** ↓ levels.
- **QT drugs, sedatives, alcohol:** caution.

NURSING MANAGEMENT

- Postural BP and falls precautions during titration.
- Metabolic monitoring.
- Mood monitoring; suicide risk.
- Watch for misuse (sedation).

PATIENT EDUCATION

- Take in the evening; may cause drowsiness.
- Rise slowly.
- Avoid alcohol and grapefruit.
- Report fever, sore throat, palpitations.

KEY POINTS

- Atypical — very low EPS; sedating.
- Titrate; postural hypotension.
- Bipolar depression and mania.
- CYP3A4 interactions; metabolic effects.